

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 65 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: September 15, 2010 | Provider: Eleanor Park, MD — Internal Medicine | Doc ID: RFH-2010-0915-NP

New Patient — Comprehensive History & Physical

Establishing Care / Annual Health Maintenance

Chief Complaint

65-year-old man establishing primary care after relocation. No acute complaints. Requests baseline evaluation and 'a full check-up.'

History of Present Illness

Mr. Whitmore is a 65-year-old retired postal supervisor who reports generally feeling well. He notes mild exertional fatigue climbing two flights of stairs but no chest pain, orthopnea, or edema. He has known borderline-high blood pressure noted at pharmacy screenings over the past two years, never treated. He has gained roughly 7 kg over the last 5 years. Reports increased thirst and nocturia (x2-3) over recent months. No polyuria, visual change, or weight loss.

Past Medical / Surgical History

Borderline hypertension (untreated). Appendectomy age 22. No prior hospitalizations otherwise. No known diabetes or coronary disease.

Social History

Cigarettes: ~1 pack/day since age 25 (~40 pack-years), currently smoking. Alcohol: 3-4 beers/week. Retired. Lives with spouse. Sedentary.

Family History

Father — myocardial infarction at 68. Mother — type 2 diabetes. Brother — prostate cancer at 74.

Allergies

No known drug allergies.

Current Medications

Medication	Dose	Route	Frequency	Indication
Aspirin (OTC, intermittent)	81 mg	PO	PRN	Patient self-directed
Multivitamin	1 tab	PO	Daily	Supplement

Vital Signs

BP (mmHg)	HR (bpm)	RR	Temp	SpO2	Weight	BMI
158/94	82	16	36.7 C	97% RA	96.4 kg	31.4

Physical Examination

General: overweight, well-appearing, no distress. HEENT: unremarkable, no carotid bruit. Cardiac: regular rate/rhythm, no murmur, S1/S2 normal. Lungs: clear, no wheeze. Abdomen: soft, obese, non-tender. Extremities: no edema, distal pulses 2+. Neuro: grossly intact. Feet: intact sensation to monofilament, no ulcers.

Assessment

- Hypertension, newly confirmed in clinic (BP 158/94) — Stage 2.
- Suspected impaired glucose metabolism — polydipsia/nocturia; screen for diabetes.
- Dyslipidemia — to be characterized.
- Tobacco use disorder, ~40 pack-years — active.
- Obesity (BMI 31.4).

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Plan

- Ordered today (results pending, fasting): CBC, comprehensive metabolic panel, HbA1c, fasting lipid panel, TSH, urinalysis. ECG performed in clinic — normal sinus rhythm, no acute changes.
- Start lisinopril 10 mg PO daily for hypertension; home BP log.
- Smoking cessation counseling provided; offered nicotine replacement — patient will 'think about it.'
- Lifestyle: DASH-style diet, 150 min/week activity, weight loss goal.
- Age-appropriate screening discussed: colorectal screening and PSA discussion at follow-up.
- Return in 4 weeks to review fasting labs and titrate therapy.

Medications Prescribed Today

Medication	Dose	Route	Frequency	Indication
Lisinopril	10 mg	PO	Once daily	Hypertension